Primary care follow-up, diabetes and blood pressure plus a new knee complaint

A full doctor-patient conversation and the finished note it should produce, laid out in the Sully SOAP Note template. Use it to test an AI scribe, train new scribes, or see what a complete note captures.

Patient	63-year-old woman (fictional)
Visit	Established patient, chronic care follow-up, in person
Length	About 13 minutes, 59 turns
Matching template	SOAP Note · sully.ai/templates/soap-note

A Visit transcript

Doctor	Hi Mrs. Washington, how are you?
Patient	Hanging in there, Dr. Kim. Hanging in there.
Doctor	Good. So we're here for your three-month check on the diabetes and blood pressure. Your daughter came with you today?
Daughter	Hi. I'm Angela. I just like to come and listen, she doesn't tell me everything.
Patient	*(laughs)* I tell her enough.
Doctor	Happy to have you, Angela. Let's start with the sugar. Your A1c came back this morning at 7.9. Last time it was 8.6.
Patient	It went down?
Doctor	It went down. That's a good drop.
Patient	I've been walking. Angela got me one of those step counters.
Daughter	She does like four thousand a day now. Before it was nothing.
Doctor	That shows. How are you doing with the metformin?
Patient	I take it twice a day with food. My stomach was bad at first, but it's fine now.
Doctor	And the Jardiance?
Patient	The new one? I take it in the morning. I have to pee more.
Doctor	That's expected. Any burning when you pee, or itching or yeast infections?
Patient	No.

Doctor	Are you checking your sugars at home?
Patient	Mornings, mostly. They're like 130, 140. One time it was 190, after Thanksgiving at my sister's.
Doctor	*(laughs)* That one's allowed. Any lows? Shaky, sweaty, confused?
Patient	No.
Doctor	OK. Blood pressure today was 142 over 86. What are you getting at home?
Patient	Like 135, 138 on top.
Daughter	She doesn't always take the amlodipine. She says it makes her ankles swell.
Patient	It does make my ankles swell.
Doctor	OK, that's important. How often are you skipping it?
Patient	Maybe half the time. When I'm going out, I don't take it, because my shoes don't fit.
Doctor	That makes total sense, and it's a known side effect. Let's fix that rather than have you skip it. Anything else bothering you?
Patient	My right knee. It's been killing me for a couple of months. Especially on the stairs, and getting up from a chair.
Doctor	Did you injure it?
Patient	No, it just started.
Doctor	Any swelling, redness, warmth?
Patient	It swells a little at the end of the day. Not red.
Doctor	Does it lock or give way?
Patient	It feels like it might give way, on the stairs. It hasn't.
Doctor	What have you taken for it?
Patient	Aleve. Two in the morning.
Doctor	Every day?
Patient	Most days.
Doctor	OK. I'm glad you told me, because with your kidneys and blood pressure, I'd rather you not take Aleve every day. We'll talk about that. Any chest pain, shortness of breath, numbness in your feet?
Patient	My feet tingle a little at night. Both of them.
Doctor	Alright. Let me examine you. *(exam)* Heart sounds good, lungs are clear. Let me look at your feet. Can you feel this? *(monofilament)* Here? Here?
Patient	Yes. Yes. That one, a little less.
Doctor	OK, slightly reduced on the bottom of both big toes. Pulses are good. No sores. Now the knee. Bend it for me. *(pause)* There's some crackling when it moves and a little fluid. Tender along the inside of the joint. Ligaments feel stable.
Doctor	OK, so let me put it together. The diabetes is moving the right way. A1c 7.9 from 8.6. Our goal for you is around 7, so we're close. I'd like to keep the medicines the same and keep up the walking. We'll recheck in three months.

Patient	OK.
Doctor	The tingling in your feet is likely early nerve changes from diabetes. Keeping sugar down helps. Check your feet every night, wear shoes that fit, and I'm sending you to podiatry for a yearly foot check.
Daughter	I can check her feet.
Doctor	Perfect. For blood pressure, I'm going to stop the amlodipine because of the ankle swelling and switch you to losartan, 50 milligrams once a day. It also protects the kidneys, which is a bonus with diabetes. Your kidney function last time was fine, but there was a little protein in the urine, so this is a good choice. I want blood work in two weeks to check your potassium and kidney numbers after the switch.
Patient	OK. One pill a day?
Doctor	One a day, in the morning with the others.
Doctor	For the knee, I think this is arthritis. I'd like an X-ray to confirm. Please stop the Aleve. Instead, try a cream called diclofenac gel. You rub it on the knee up to four times a day. It works locally, so it's much easier on your kidneys. Tylenol is fine too. And I'll send you to physical therapy. Strengthening the thigh muscles really helps knee arthritis.
Patient	Do I need surgery?
Doctor	Not at this point. If PT and the cream don't help, we can talk about an injection. We'll take it step by step.
Patient	OK.
Doctor	Last thing, you're due for your eye exam for diabetes, and you haven't had your pneumonia shot. Can we do the shot today?
Patient	Fine. One shot.
Doctor	One shot. *(laughs)* Angela, any questions?
Daughter	No, that's everything. Thank you for catching the Aleve thing.
Doctor	Of course. I'll see you both in three months.

B Finished note

Patient: Dorothy Washington (fictional), 63 F. Established patient. Accompanied by daughter (Angela), with patient's permission.

Subjective

Follow-up of type 2 diabetes and hypertension.

- **Diabetes:** Walking ~4,000 steps/day. Metformin twice daily with food, GI symptoms resolved. Empagliflozin daily, increased urination; denies dysuria, genital itching or yeast infection. Home fasting glucose 130 to 140 mg/dL, one reading of 190 after a holiday meal. No hypoglycemia. Bilateral nocturnal foot tingling.
- **Hypertension:** Home SBP 135 to 138. Taking amlodipine ~50% of days due to ankle swelling.
- **New: right knee pain** × 2 months, no trauma. Worse with stairs and rising from chair. Mild end-of-day swelling, no erythema. Sense of instability on stairs, no locking. Taking naproxen 440 mg most days.
- **Medications:** Metformin, empagliflozin, amlodipine (intermittent), naproxen OTC.

Objective

- BP 142/86.
- A1c 7.9% (prior 8.6%).
- Cardiac: regular rate and rhythm, no murmur. Lungs: clear.
- Feet: monofilament sensation slightly reduced at plantar first toes bilaterally. Pedal pulses intact. No ulcers.
- Right knee: crepitus, small effusion, medial joint line tenderness, ligaments stable.
- Prior labs: eGFR normal; urine albumin-to-creatinine ratio mildly elevated.

Assessment

1. Type 2 diabetes, improving, not at goal (A1c 7.9%, target ~7%).
2. Early diabetic peripheral neuropathy, bilateral.
3. Hypertension, not at goal; amlodipine-related ankle edema with nonadherence.
4. Moderately increased albuminuria.
5. Right knee pain, likely osteoarthritis.
6. Daily NSAID use with diabetes, hypertension and albuminuria.

Plan

1. Continue metformin and empagliflozin. Continue walking. Repeat A1c in 3 months.
2. Daily foot checks (daughter to assist); podiatry referral for annual diabetic foot exam.
3. Stop amlodipine. Start losartan 50 mg daily. BMP (potassium, creatinine) in 2 weeks.
4. Stop naproxen. Topical diclofenac gel up to 4 times daily; acetaminophen as needed.
5. Right knee X-ray (weight-bearing). PT referral for quadriceps strengthening. Consider injection if no improvement.
6. Diabetic eye exam referral. Pneumococcal vaccine given today.
7. Follow-up 3 months.

How to use this sample. Run the transcript through your scribe and compare its draft with the finished note. Check the details that are easy to miss: doses, which side of the body, what the patient said no to, and every follow-up step. For 536 more transcripts, many with audio and reference notes, see github.com/aniquethaplawala/open-clinical-encounters.

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